

SUDAN

Cholera Operational Update

3 July 2025

A child receives an oral cholera vaccine in Gedaref state. Photo: WHO/Omer Tarig

HIGHLIGHTS

- In 2025 alone, over **32,000** suspected cholera cases have been reported in Sudan, fuelling a total of more than **83,000** cases and **2,100** deaths since the outbreak began in July 2024, according to the Federal Ministry of Health. The disease continues to spread across the country amid conflict and collapsing infrastructure.
- More than **33.5** million people – including **5.7** million children under five – are at risk, with rising cases in Darfur and reports of cross-border transmission into Chad and South Sudan.
- Urgent funding of **\$50** million is needed to sustain the ongoing cholera response. The response plan is currently funded at 16 per cent, but more funds are needed to scale up the response. Without additional support, cases and deaths will rise, supplies will run out, and the outbreak risks further compounding the already dire humanitarian situation.

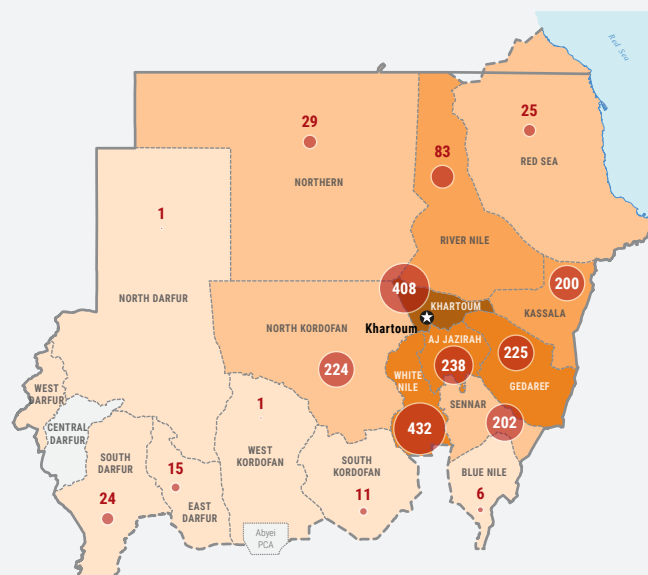
SITUATION OVERVIEW

Sudan's cholera outbreak is worsening, driven by conflict, displacement and a collapsed public health system. Since July 2024, over **83,000** cases and **2,100** deaths have been reported. The crisis escalated in 2025, with over **32,000** new suspected cases already documented this year alone. The outbreak remains widespread, affecting 16 out of 18 states.

Aj Jazirah, Gedaref, Khartoum and White Nile states represent over 70 per cent of all reported cases, with Khartoum alone accounting for more than **22,000** suspected cases. Years of underinvestment, compounded by the destruction of water treatment plants, health centres and power infrastructure, have left millions without access to safe water, basic sanitation or health care. In Khartoum, 72 per cent of health facilities are non-functional.

States in the Darfur region continue to report a sharp increase in suspected cases. The situation is compounded by ongoing conflict and deteriorating humanitarian access. Many areas remain cut off due to insecurity, limiting assessments and service delivery. As the rainy season continues until October, there is a heightened risk of flooding, which could further contaminate water sources and intensify the outbreak.

Cholera cases/deaths by state



Country capital Cumulative number of deaths per state

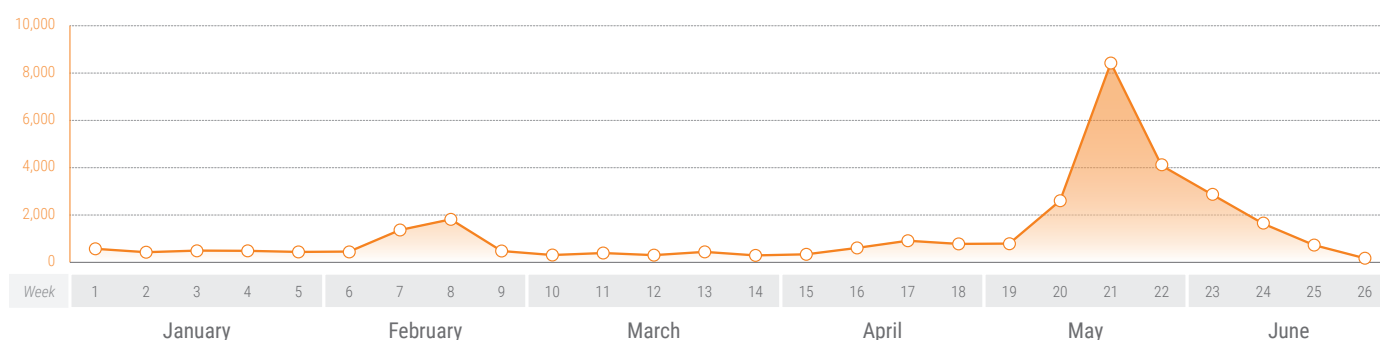
Cumulative number of cases by state

10 - 500 1,000 - 5,000 6,000 - 10,000 11,000 - 13,000 > 22,000

Source: General Directorate of Health Emergencies & Epidemics Control.



Cholera cases trend



HUMANITARIAN RESPONSE

The cholera response in Sudan is being delivered under an integrated and coordinated plan that combines health, water, sanitation and hygiene (WASH), risk communication, logistics and vaccination efforts under a unified operational framework. This approach ensures that interventions reinforce each other and reach communities with comprehensive and life-saving support.

1. Surveillance, early detection and case management

Responses: To strengthen outbreak surveillance and ensure rapid detection of cases, the World Health Organization (WHO) has expanded the Early Warning, Alert and Response System (EWARS) to 573 health facilities in Darfur, supported by 28 implementing partners. WHO has also deployed seven public health officers to Khartoum and trained staff in water quality monitoring and community-based surveillance techniques.

UNICEF, WHO and their partners are supporting over 700 health facilities across Sudan. Four new oral rehydration points (ORPs) are currently operational, with an additional 52 under establishment. Cholera treatment centres (CTCs) and cholera treatment units (CTUs) have been reinforced, with health cluster partners providing essential services, supplies and technical assistance.

Challenges: Reporting delays and limited laboratory capacity hinder early case confirmation. Insecurity, shortages of trained staff and limited functional infrastructure in high-burden areas continue to affect the quality and timeliness of treatment.

Priorities: Maintaining and expanding CTCs, CTUs and ORPs, training frontline health-care workers, improving case tracking and reporting systems, and reinforcing mobile teams for early detection and timely care.

2. Water quality, sanitation and infection prevention

Responses: Providing safe water and proper sanitation is central to stopping the spread of cholera. Millions of people have benefited from emergency WASH support. In Darfur, UNICEF has delivered clean water to 2.5 million people via chlorination, trucking and expansion of storage and pumping capacity.

Urban water systems in Khartoum and White Nile are being repaired and solarized to improve reliability. Local maintenance teams are also being trained to manage and sustain infrastructure. Emergency chlorination and wastewater treatment have been intensified at both the community and household levels.

Challenges: One in four water sources remains unsafe, and over a third of household water samples are contaminated. Resistance to water chlorination due to misinformation, coupled with poor access in insecure areas, continues to hinder WASH efforts.

Priorities: Replenishing hygiene and water treatment stocks, rehabilitating and solarizing systems, expanding chlorination and improving testing and sanitation, particularly in IDP camps.

3. Risk communication and community engagement (RCCE)

Responses: Reaching communities with trusted information remains a priority. More than 2 million people have been engaged through community leaders, radio, mobile teams and information, education and communication (IEC) materials.

RCCE campaigns promote handwashing, safe water use, food hygiene and early health-seeking behaviour. Efforts are being made to integrate vaccine messaging into outreach activities to boost acceptance. Religious and traditional leaders are being enlisted to dispel rumors and improve uptake of cholera-related services.

Challenges: Gaps remain in outreach coverage, particularly in rural and conflict-affected areas. Only 1.5 per cent of cholera cases had prior vaccination, underscoring the urgent need to expand public education.

Priorities: Expanding outreach to underserved areas, countering misinformation, building trust in vaccines and water safety, and supporting localized campaigns through community influencers.

4. Vaccination and outbreak containment

Responses: Vaccination is a critical line of defense. More than 3 million oral cholera vaccine (OCV) doses have been delivered, with 1.3 million people vaccinated in Khartoum. A further 3 million doses are in the pipeline for other high-risk states.

The vaccination campaign is being rolled out alongside RCCE efforts to counter hesitancy and misinformation. Vaccinators are being trained to communicate safety and effectiveness messages during household visits. Maintaining cold chain systems and expanding vaccine delivery to underserved areas remain key priorities.

Challenges: While coverage has increased in high-risk areas, delays in campaign rollout, vaccine hesitancy and logistical gaps continue to limit scale and impact.

Priorities: Procuring and distributing more OCV doses, expanding coverage to new hotspots, integrating campaigns with RCCE and supporting cold chain systems and logistics for rapid vaccine deployment.

5. Logistics and supply chain support

Responses: Efficient logistics are the backbone of the response. Over 600 metric tons of essential supplies have been delivered across eight states, including cholera kits, oral rehydration salts (ORS), intravenous fluids and water treatment chemicals. WHO and UNICEF continue cross-border operations and in-country supply chain support.

Challenges: Delays at Port Sudan, high transportation costs, fuel shortages, visa issues and insecurity continue to slow delivery. Stockouts of critical supplies jeopardize the continuity of essential services in hard-to-reach areas.

Priorities: Ensuring the timely delivery of cholera kits, ORS, IV fluids, and water treatment supplies, expanding last-mile delivery networks, and addressing customs, fuel and access bottlenecks to maintain consistent supply flow.

6. Coordination and strategic planning

Responses: Cholera response efforts are coordinated through joint platforms led by WHO, UNICEF and the Federal Ministry of Health. Coordination forums and taskforces are active in Khartoum and across affected states, with regular cluster meetings facilitating partner engagement, needs assessments and data sharing. Cross-border coordination with South Sudan is also being strengthened to reduce regional transmission risks.

Challenges: Gaps in field reporting, inconsistent data flow and fragmented coordination mechanisms limit the effectiveness of response planning and execution.

Priorities: Bolstering subnational response operations, strengthening cross-border coordination, harmonizing data systems and enhancing decision-making at field level to ensure a cohesive and agile response.

FUNDING

To sustain and scale up cholera response operations through the end of 2025, humanitarian partners require **\$50 million**.

During the first half of the year, pooled funds proactively allocated \$8 million to support cholera preparedness and early response efforts – including **\$3 million** from the Central Emergency Response Fund and **\$4.9 million** from the Sudan Humanitarian Fund (SHF).

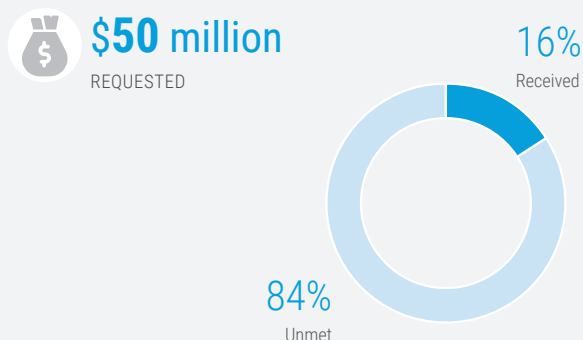
In addition, they responded to active cholera outbreaks with a further **\$5.4 million** in targeted allocations, enabling rapid interventions across affected areas. These flexible resources enabled the rapid establishment of oral rehydration points and cholera treatment centres, hygiene promotion campaigns, vaccine deployment, clean water access and mobile surveillance teams.

The SHF is currently reprogramming an additional \$5 million towards health activities with a strong focus on cholera response. Moreover, **\$2.87 million** in prepositioned SHF funding remains available with health and WASH cluster partners for immediate use. A further **\$3 million** in contingency funding has been reserved by SHF for NGOs operating in North and South Darfur, who are now assessing the best use of these funds. Altogether, SHF has over **\$10 million** immediately available for NGOs for cholera response activities.

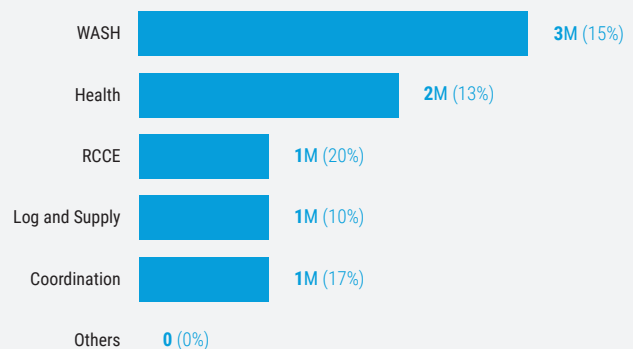
Pooled funds are also exploring further allocations in the coming weeks to bolster response efforts during the rainy season.

However, additional contributions are urgently needed to sustain and expand operations, particularly as needs intensify and new hotspots emerge. Flexible and timely funding is critical to maintaining momentum. Without it, gains may be reversed and the outbreak could escalate – within and beyond Sudan's borders.

Cholera preparedness and response plan



Funding by pillar (US\$)



Footnote: Exact pillar allocations may vary and are intended to complement each other.